

Patient ID: Harris

Period: April 1 to April 30, 2026

Report Date: May 1, 2026

Coverage: 659/720h (92%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 30 days from Apr 01 to Apr 30

■ HR ■ Breathing

Elevated Heart Rate Apr 06 to Apr 07 (2d)	Elevated Heart Rate Apr 13 to Apr 15 (3d)	Elevated Breathing Apr 21 to Apr 21 (1d)	High Heart Rate Apr 07 to Apr 07 (1d)	High Heart Rate Apr 26 to Apr 26 (1d)
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Last 24 Hours

Vital	Avg	Min	Max
Heart Rate	77 bpm	48 bpm	109 bpm
Breathing	17 brpm	12 brpm	40 brpm

12 episodic events Detected, spanning 20h total. Conditions: Elevated Breathing, Elevated Heart Rate, High Heart Rate. **Priority grade: Medium**

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
12	20h	Elevated Heart Rate	<1	92%

Major Findings: Sustained elevated HR (avg 98, peak 111)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Apr 07	12 AM to 1 AM	1h	1	High Heart Rate	104	80	110	Check temp, hydration, pain; review for arrhythmia or infection
Apr 26	1 AM to 2 AM	1h	1	High Heart Rate	100	87	111	Check temp, hydration, pain; review for arrhythmia or infection
Apr 06	7 PM to 12 AM	11h	5	Elevated Heart Rate	98	72	109	Check temp, hydration, pain; review for arrhythmia or infection
Apr 13	1 AM to 4 AM	6h	4	Elevated Heart Rate	97	54	104	Check temp, hydration, pain; review for arrhythmia or infection
Apr 21	11 AM to 12 PM	1h	1	Elevated Breathing	24	18	35	Check SpO2 and lung sounds; assess for fluid overload or early infection

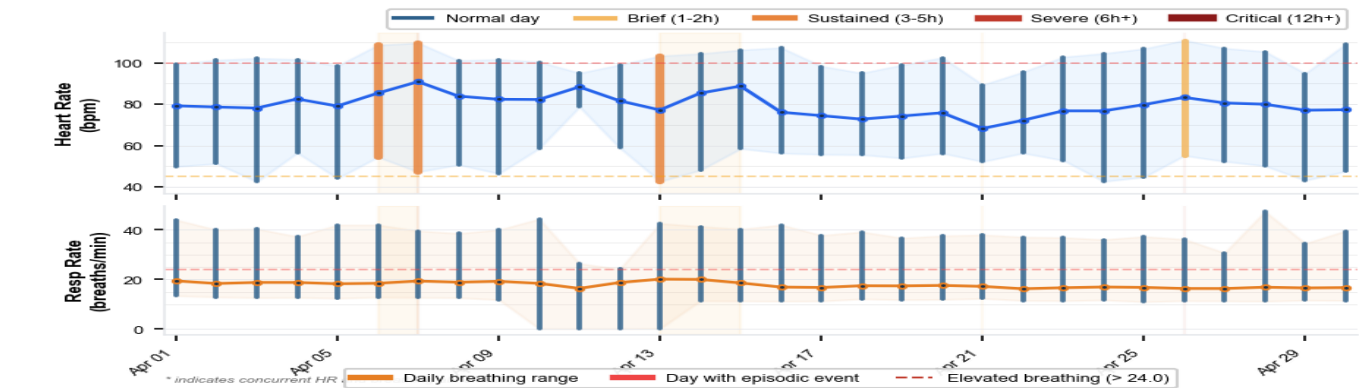
The P5 to P95 heart rate spread was 28 bpm (65 to 93), indicating significant cardiac variability.

Suggested Clinical Review Actions

- Elevated Heart Rate (avg 98 bpm, peak 109 bpm): Check temp, hydration, pain; review for arrhythmia or infection
- Elevated Breathing (avg 24 brpm, peak 35 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- High Heart Rate (avg 104 bpm, peak 110 bpm): Check temp, hydration, pain; review for arrhythmia or infection
- Overall trajectory shows 5 unstable phases with 28 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

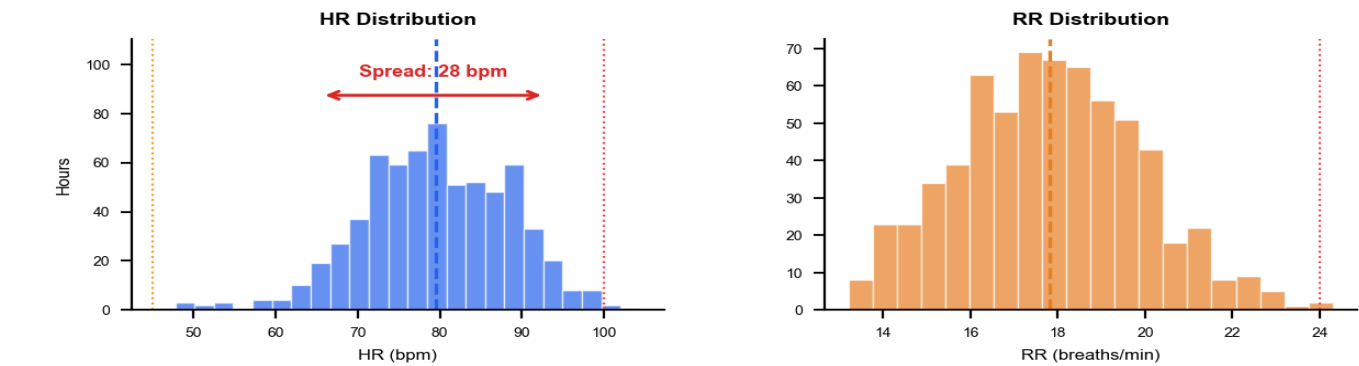
Clinical Pattern Observations:

- 1. High variability: HR spread 27 bpm (65 to 93).
- 2. Clustered pattern: Episodic events on 4 of 30 days (13%).
- 3. Nocturnal pattern: 5 of 6 eligible episodes during evening or night hours.

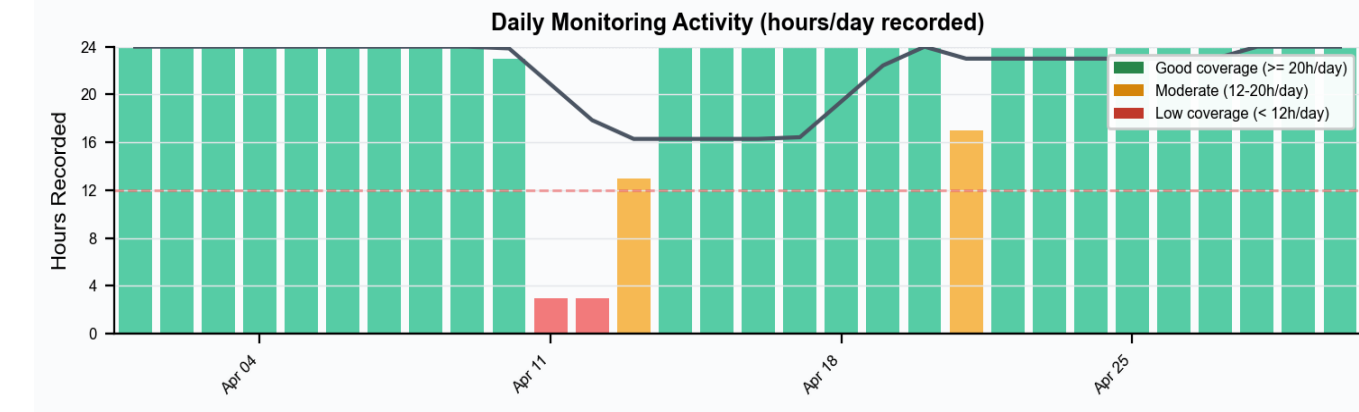


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

- Very Low HR < 40 bpm
- Low HR < 45 bpm
- Elevated HR > 95 bpm
- High HR > 100 bpm
- Very High HR > 110 bpm
- Elevated Breathing > 24 bpm
- High Breathing > 30 bpm
- Very High Breathing > 40 bpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.